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Medicare⁺

MediGESIC TABLET 450 MG / 35 MG

VIXXxx-08

DESCRIPTION

Round, white uncoated tablet, bevel-edged, flat faces, "HD" embossed and scored on the same face. The tablet is not meant to be split and should be swallowed whole as the break line is for solely for identification purposes.

COMPOSITION

Each tablet contains:
Paracetamol 450 mg
Orphenadrine citrate 35 mg

PHARMACODYNAMICS

Orphenadrine is a skeletal muscle relaxant.
Paracetamol is an analgesic and antipyretic.

PHARMACOKINETICS

Orphenadrine is readily absorbed from the gastrointestinal tract and is almost completely metabolised to at least eight metabolites. Orphen-adrine and its metabolites are excreted from the body in the urine, with a half life of 14 hours.

Paracetamol is also readily absorbed from the gastrointestinal tract with peak plasma concentrations occurring about 10 to 60 minutes after oral administration. Paracetamol is distributed into most body tissues. It has a half life of between 1 to 3 hours.

INDICATION

Tension headache, occipital headaches associated with spasm of skeletal muscles in the region of the head and neck. Acute and traumatic conditions of the limbs and trunk: sprains, strains, whiplash injuries, acute torticollis, prolapsed intervertebral disc.

CONTRAINDICATIONS

Orphenadrine shows some anticholinergic activity and should not be used in patients with glaucoma, prostatic hypertrophy or obstruction at the bladder neck, or myasthenia gravis.

WARNING AND PRECAUTIONS

Orphenadrine may impair the ability of the patient to engage in potentially hazardous activities such as operating machinery or driving a motor vehicle; ambulatory patients should therefore be cautioned accordingly.

Orphenadrine citrate should be used with caution in patients with tachycardia, cardiac decompensation, coronary insufficiency, cardiac arrhythmias.

Safety of continuous long-term therapy with orphenadrine has not been established. Therefore if orphenadrine is prescribed for prolonged use, periodic monitoring of blood, urine and liver function is recommended.

**This preparation contains PARACETAMOL.
Do not take any other paracetamol containing
medicines at the same time.**

Allergy alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

EFFECTS OF ABILITY TO DRIVE AND USE MACHINE

Be careful driving or operating machinery as MediGESIC may cause some people to have blurred vision or to become drowsy. If you have any of these symptoms do not drive or operate machinery or do anything else that could be dangerous.

PREGNANCY AND LACTATION

Safe use in pregnancy has not been established, therefore the drug should not be used in pregnant women or those likely to become pregnant unless the expected benefits outweigh the potential risks.

It is unknown whether orphenadrine is excreted during lactation. However it is established that paracetamol is excreted into the breast milk, one to two hours after oral administration.

SIDE EFFECTS

Side effects rarely occur at the recommended dosage. Those encountered are associated withanticholinergic activity and may include nausea, dry mouth, blurring of vision. Rarely, rash or drowsinessmay occur. These symptoms disappear rapidly with a reduction in dosage or cessation of medication. Notoxic effects have been observed.Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.

DRUG INTERACTIONS

The effects of antimuscarinic agents such as orphenadrine maybe enhanced by other drugs with antimuscarinic properties such as amantadine, some antihistamines, butyrophenones and phenothiazines, and tricyclic antidepressants. The reduction in gastric motility caused by antimuscarinic agents may affect the absorption of other drugs.

OVERDOSAGE AND TREATMENT

Symptoms

Symptoms of orphenadrine overdosage are excite-ment, confusion, delirium leading to coma. Convulsions and tachycardia with dilated pupils and urinary retention may occur. Paracetamol overdosage may cause acute liver damage, but symptoms may not appear for up to several days after ingestion.

Treatment

Gastric lavage should be carried out immediately regard-less of the estimated ingested dose. Convulsions and delirium respond to relatively large doses of diazepam, preferably by mouth. Adequate hydration of the patient is important. It is recommended that the patient be referred to a hospital where early and regular monitoring of plasma paracetamol levels can be carried out. If instituted sufficiently early, treatment with N-acetylcysteine, L-methionine or L-cysteamine will minimise liver damage.

DOSAGE AND ADMINISTRATION

2 tablets three times daily.

Note: The information given here is limited. For further information consult your doctor or pharmacist.

Storage:

Store below 30°C. Protect from light and moisture.

Presentation/Packing:

PVDC blister of 10 x 10's.

Product Registration Holder:

ALLPAC SOLUTIONS SDN.BHD.
121, Jalan Tunku Abdul Rahman
(Jalan Kuala Kangsar),
30010 Ipoh, Perak, Malaysia.

Manufactured by:

HOVID Bhd.
Lot 56442, 7 ½ Miles, Jalan Ipoh/Chemor,
31200, Chemor, Perak, Malaysia.

Marketed by:

World Medicare Supplies Sdn. Bhd.
PT 5740 (1st Floor), Jalan TS 2/1E,
Taman Semarak 2, 71800 Nilai,
Negeri Sembilan, Malaysia.

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